

Faculty of Medicine

Department of Medicine

Module : Health, Society and Humanity – Chapter III: Ethics,
Deontology, Law and Health

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Course Title: BIOETHICS

Objectives :

At the end of the course, the learner should be able to:

- Define bioethics.
- Recognize all ethical measures related to activities concerning:
 - *therapeutic obstinacy (futile medical care),
 - *euthanasia,
 - *organ transplantation and grafts,
 - * and medically assisted reproduction.
- Identify the limits regarding the respect of human dignity in healthcare.

Course Outline :

I. Introduction – Definition

II. Bioethics Committees Around the World

III. The Dignity of the Human Person

IV. Analysis of Ethical Issues Related to Biomedicine (Bioethics Topics):

A. Reflections on Questions of Life and Death:

a. Therapeutic obstinacy

b. Euthanasia

B. The Field of Medically Assisted Reproduction (MAR)

C. The Field of Organ Procurement

V. Conclusion

VI. Bibliographic References

VII. Legal Texts

I/ Introduction – Definition :

A profession that, for thousands of years, has dealt with life—particularly human life—is medicine.

But today, many scientific disciplines and professions intervene in this field.

Medically assisted reproduction, end of life, neurosciences, and predictive medicine... all these areas share the fact that they lie at the heart of current issues raised by bioethics. But what exactly is bioethics?

The term bioethics was used for the first time in 1970 by Van Rensselaer Potter (an oncologist at the University of Wisconsin), who defined bioethics very broadly as “the science of survival.”

Potter created the word bioethics by combining two Greek words: *bios*, life—which represents living reality and the life sciences—and *ethos*, morality, which refers to values and duties.

The main objective of bioethics is to conduct a rational analysis of the moral problems related to the evolution of biomedicine while respecting the values of the individual (religious, sociocultural beliefs), thus involving several disciplines (philosophy, theology, sociology, law, biology, medical sciences...).

- Consequently, bioethics should not be confused with medical ethics or medical deontology.

Bioethics is a reflection on advances in research in the fields of biology, medicine, and health.

It is therefore a collaborative effort at the crossroads of several disciplines: science, philosophy, law, and medicine.

Article 354 of Law n°18-11 of 02/07/2018 on health (LRS) defines bioethics as “all measures related to activities concerning:

- The transplantation and grafting of organs, tissues, and cells,
- The donation and use of human blood and its derivatives,
- Medically assisted reproduction,
- And biomedical research.”

II – Bioethics Committees Around the World

1 – At the Maghreb level

• Algeria:

Creation of the National Council of Ethics for Health Sciences by Law 85-05 of 16/02/1985, completed by Law 90-17 of 31/07/1990 in its Article 168/1, established in October 1996.

Law n°18-11 of July 2, 2018, on health reaffirms in Title VII the principles of ethics, deontology, and bioethics.

The same law defines bioethics in Article 354 as “all measures related to activities concerning organ, tissue, and cell transplantation and grafting, donation and use of human blood and its derivatives, medically assisted reproduction, and biomedical research.”

This National Council of Ethics and Health Sciences is composed of:

- 1 representative of the Ministry of Health
- 9 professors in medical sciences
- 3 healthcare practitioners
- 1 representative of the Ministry of Justice
- 1 representative of the Islamic Council
- 1 representative of the National Council of Medical Deontology

- Tunisia:

Established a National Committee for Medical Ethics by decree in September 1994.

- Morocco:

Adopted Law 28-13 in 2015. Bioethics committees for biomedical research have been implemented in university hospitals and medical faculties. However, there is still no national bioethics committee.

2 – In other countries around the world

National ethics committees were created around the 1970s in the USA, Spain, the UK... and in the 1980s in the Netherlands, France...

III – Human Dignity

In contemporary international law, the concept of human dignity is closely linked to human rights.

According to Article 1 of the Universal Declaration of Human Rights (1948) :

“All human beings are born free and equal in dignity and rights.”

The Declaration bases rights on the inherent dignity of every human being.

Unlike material goods and financial values, human dignity has no external equivalent; it is an end in itself.

All human beings are equal in dignity regardless of sex, age, social status, or ethnicity.

Recognizing a person's dignity therefore presupposes active respect for their human rights, self-esteem, and autonomy; it also implies

protecting their privacy from unlawful intrusion and preserving their place in public life.

This notion further requires that the interests and well-being of the individual take precedence over “the sole interest” of society, science, or the community.

Emphasizing “the sole interest of science or society” is important:

it means that, because of their human dignity, the individual must never be sacrificed in the name of science or society.

However, the term “sole” also implies that exceptional circumstances may exist in which the interest of others, or of the entire community, is so important that some infringement upon the interests of individuals becomes inevitable for the purpose of saving other persons or the community as a whole.

From a bioethical perspective, dignity and human rights require that others treat the individual with respect, meaning not harming them, not abusing them, being fair, not imposing unwanted notions of good or happiness, not treating them as a mere means, and not subordinating their interests and well-being to those of others or to “the sole interest of science and society.”

IV – Analysis of Some Moral Issues Related to Biomedicine (Bioethics Topics)

1 – Reflection on Life and Death Issues :

In the name of the sanctity of life, medicine has long favored therapeutic obstinacy, which has led to numerous court cases and advocacy for euthanasia.

a-Therapeutic obstinacy : is defined as the implementation or continuation of heavy curative treatments or technical interventions when the patient is in a condition where all hope of recovery or remission is lost.

b-Euthanasia is defined as the act of a third party deliberately ending the life of a person in order to end a situation deemed unbearable.

We distinguish:

- **Active euthanasia:** administering a lethal medication to a patient suffering greatly and who is condemned anyway;
- **Passive euthanasia:** stopping ongoing treatments, knowing that this will hasten the patient's death.

In Algeria, Muslim ethics and the law do not allow assisted suicide or euthanasia.

In France, the general approach is to favor palliative care and prohibit active euthanasia.

However, some countries allow active euthanasia, passive euthanasia, and assisted suicide, such as Switzerland, some U.S. states, Spain, Belgium, Luxembourg, the Netherlands, Germany, and Colombia.

The difference in viewpoints between supporters of euthanasia and defenders of therapeutic obstinacy is often summarized as the opposition between the principles of life and death.

Faced with an end-of-life patient, the caregiver encounters a conflict between two fundamental and contradictory ethical values: respecting life (leading to fighting death at all costs and rejecting euthanasia) and respecting human dignity.

The patient should not be treated as an object whose life must be prolonged solely to relieve the caregiver's guilt.

The refusal of therapeutic obstinacy is based on three principles:

Do not continue useless treatments;

Ensure proportionality between the expected benefits for the patient and the aggressiveness of the treatment;

Respect the patient's human dignity.

However, stopping aggressive therapy does not mean abandoning care.

Palliative care aims to accompany the patient, to continue relieving physical, psychological, and moral suffering, and to improve quality of life.

Although it may be difficult for the caregiver to transition from curative to palliative care, this is essential for the well-being and dignity of the dying person.

2 – Medically Assisted Reproduction (MAR / PMA)

1 – Definition :

MAR consists of clinical, biological, and therapeutic practices allowing ovulation stimulation, in vitro fertilization, embryo transfer, and artificial insemination (Article 370 of the LRS).

Its purpose is to remedy medically diagnosed pathological infertility or to prevent the transmission of a serious hereditary disease.

Legally accepted methods consist of insemination with the spouse within the couple (IVF) (Article 371 of the LRS).

2 – Deontological Principles :

The request must be made by a couple consisting of a man and a woman, alive, of reproductive age, married for at least two years, and consenting to the proposed technique.

Obstacles include:

- death of one member of the couple,
- filing for divorce,
- withdrawal of consent by either partner.

Ethical and deontological recommendations regarding MAR are based on the following principles:

- Assist the couple;
- Respect the embryo from conception: creating an embryo for study, research, donation, sale, experimentation, or commercial/industrial purposes and any form of transaction is strictly prohibited (Article 374 of the LRS).
- Consider the child to be born (specific provisions must be taken for couples at viral risk) : Article 373 of the LRS.

3 – The Field of Organ Removal

Organ transplantation is an unparalleled medical solution to a major public health problem, given that Algeria currently has 14,000 patients with kidney failure, 500 with liver failure, more than 1,000 requests for corneal transplants, and 250 awaiting lung transplants (SATO, 2011).

Organ and tissue removal for transplantation may be performed on living or deceased donors (brain death).

Interest:

- Scientific (diagnosis, determining cause of death in scientific autopsy);
- Therapeutic (for the recipient).

Risks :

- Organ trafficking and sales, a new form of slavery (the body is inviolable and not for commerce);
- Organ theft.

*Ethical principles and regulatory conditions must be observed:

1-These organ removals may be performed only for therapeutic or diagnostic purposes and must be the only possible medical option to treat a patient in need (Article 355 of the LRS).

2-Donations must be free of charge:

Organ and tissue removal and transplantation may not involve any financial transaction (Article 358 of the LRS).

3-No remuneration may be received by practitioners performing organ transplants (Article 367 of the LRS).

4-Anonymity is mandatory: the identity of the deceased donor must not be revealed to the recipient or the recipient's identity to the donor's family (Article 363 of the LRS).

5-Medical rules must be observed: screening for transmissible diseases by detecting biological markers of infection—HIV1, HIV2, HTLV1, HTLV2, hepatitis B and C, cytomegalovirus, Epstein-Barr virus, syphilis, toxoplasmosis—must be performed (Articles 359 and 360 of the LRS).

6-Clear information must be provided to the donor and recipient concerning the risks involved, the possible consequences of the removal and transplantation, and the expected results for the recipient (Articles 360 and 364 of the LRS).

7- Informed and written consent from both donor and recipient is required. At any time, the donor may reconsider and withdraw their prior consent (Articles 360 and 364 of the LRS).

VI – Conclusion :

Bioethics calls for reflection on potential “drifts” that advances in medicine, research, and the application of treatments or care techniques for non-medical purposes may generate.

It therefore invites us to consider the limits that must be set to ensure that humans do not harm one another.

The general idea of bioethics is that what is technically possible is not necessarily morally acceptable, and that our interventions in nature and the environment, on animals and on human beings, must be subject to certain controls.

The future of life and humanity is at stake.

VII- Bibliographic References:

1-Law No. 18-11 of July 2, 2018, relating to health. JORADP No. 46 of July 29, 2018.

2- Universal Declaration of Human Rights. United Nations, General Assembly, December 10, 1948.

3-M. Stanton-Jean. The Universal Declaration on Bioethics and Human Rights:

A Vision of the Common Good in a Global Context of Plurality and Cultural Diversity?

Dissertation submitted for the degree of Ph.D., Applied Human Sciences Program, Bioethics Option, Faculty of Arts and Sciences, Montreal, October 2010.

VIII – Legal Texts:

1. Human Dignity :

-Art. 340 of the LRS:

In the exercise of their activities, health professionals must be guided by ethical values, notably the principles of respect for human dignity, honor, equity, professional independence, rules of ethics, as well as factual consensus.

-Article 3 of the Universal Declaration on Bioethics and Human Rights:

- Human dignity, human rights, and fundamental freedoms must be fully respected.

-The interests and welfare of the individual should prevail over the sole interest of science or society.

-Article 11 of the UDBHR:

No individual or group should be subjected, in violation of human dignity, human rights, and fundamental freedoms, to discrimination or stigmatization for any reason whatsoever.

2. Medically Assisted Reproduction (MAR)

-Art. 371 of the LRS:

“Only the husband's sperm and the wife's ovum may be used, to the exclusion of any other person.”

-Art. 375 of the LRS:

“All reproduction of genetically identical living organisms involving humans, as well as any selection of sex, is prohibited.”

-Paragraph 2 of Art. 371:

“The husband and the wife must both submit, during their lifetime, a written request for medically assisted reproduction, which must be

confirmed by them after one (1) month from the date it is received by the relevant structure or establishment.”

-Art. 374 of the LRS:

“The following are prohibited: manipulation for scientific research purposes, donation, sale, and any other form of transaction involving:

— spermatozoa;

— oocytes, even between co-wives;

— surplus embryos or otherwise, to a surrogate mother or another woman, sister, mother, or daughter;

— cytoplasm.”

3. National Council for the Ethics of Health Sciences

-Art. 342:

A national council for the ethics of health sciences is hereby established under the authority of the minister in charge of health.

4. Organ Donation

-Art. 355 of the LRS:

“The removal and transplantation of human organs, tissues, and cells may only be performed for therapeutic or diagnostic purposes and under the conditions provided for in this law.”

-Art. 358 of the LRS:

“The removal and transplantation of human organs, tissues, and cells may not be the subject of any financial transaction.”

-Art. 367 of the LRS:

“No remuneration for the act may be received by practitioners performing organ, tissue, or cell removals or transplantations.”

-Art. 359 of the LRS:

“The removal and transplantation of human organs and cells may be carried out from living related donors who are compatible with the recipient, in strict compliance with medical rules.

The cost of medical examinations and the medical follow-up of both donor and recipient must be covered by the health facility to which the recipient belongs.”

-Art. 360 of the LRS:

“The removal of organs, tissues, or cells for transplantation may not be performed on living persons if such removal endangers the donor’s life. (...)

The removal of organs and cells from a living person for transplantation may not be performed without the donor’s informed consent.

The donor must express consent to the donation, and, where applicable, to cross-donation before the president of the territorially competent court, who must first ensure that the consent is free and informed and that the donation complies with the conditions of this law.

The donor may withdraw consent at any time without any formality. (...)

The donor must first be informed by a committee of experts of the risks involved, the possible consequences of the removal, and the expected results of the transplant for the recipient.”

-Art. 363:

“It is forbidden to disclose the identity of the deceased donor to the recipient and the identity of the recipient to the donor’s family.”

-Art. 364:

“The transplantation of human organs, tissues, or cells may only be performed if it is the sole means of preserving the recipient’s life or

physical integrity, after the recipient has expressed consent in the presence of the head physician of the department in which he or she has been admitted, and two (2) witnesses.

When the recipient is unable to express consent, one adult family member may express it in writing, in the order of priority specified in Article 362 above.

In the case of legally incapacitated persons, consent may be expressed by the father, mother, or legal representative, as appropriate.

In the case of minors, consent is expressed by the father, mother, or failing that, the legal representative.

Consent may only be expressed after the recipient or the persons referred to in paragraphs 3 and 4 above have been informed by the attending physician of the medical risks involved.

The transplantation of human organs, tissues, or cells may be performed without the consent provided.